

Calendar Year 2026 MI Coordinated Health Capitation Rate Certification

January 1, 2026 through December 31, 2026

State of Michigan Department of Health and Human Services

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Introduction & Executive Summary

BACKGROUND

Milliman, Inc. (Milliman) has been retained by the State of Michigan, Department of Health and Human Services (MDHHS) to provide actuarial and consulting services related to the development of actuarially sound capitation rates for the MI Coordinated Health (MICH) program to be effective January 1, 2026. The rates being certified as actuarially sound are to be effective from January 1, 2026 and remain in effect through December 31, 2026. MI Coordinated Health is Michigan's managed care program for the Highly Integrated Dual Eligible Special Needs Plan (HIDE SNP).

This letter provides documentation for the development of the actuarially sound capitation rates for calendar year CY 2026. It also includes the required actuarial certification in Appendix 1. Unless otherwise specified, all references to "rates" or "capitation rates" throughout this document refer to the Medicaid-specific component of the MICH capitation rates.

To facilitate review, this document has been organized in the same manner as the 2025-2026 Medicaid Managed Care Rate Development Guide, released by the Center for Medicare and Medicaid Services in August 2025 (CMS guide). Section III of the CMS guide is not applicable to this certification, since the covered services do not include rates for new adult groups.

In developing the capitation rates and supporting documentation herein, we have applied the three principles of regulation outlined in the CMS Guide:

- The capitation rates are reasonable and comply with all applicable laws (statutes and regulations) for Medicaid managed care.
- The rate development process complies with all applicable laws (statutes and regulations) for the Medicaid program, including but not limited to eligibility, benefits, financing, any applicable waiver or demonstration requirements, and program integrity.
- The documentation is sufficient to demonstrate that the rate development process meets the requirements of 42 C.F.R. § 438 and generally accepted actuarial principles and practices.

SUMMARY OF CAPITATION RATES

The capitation rates for the MICH program are illustrated in Table 1 by rate cell. As there is not a previous rate development for the MICH program, there is no comparison to prior capitation rates. The underlying capitation rates by rate cell are effective from January 1, 2026 through December 31, 2026. The capitation rates covered under this certification are documented in Appendix 3. The rates in Table 1 are illustrated on a gross basis prior to adjustment for any amounts that are expected to be paid by the beneficiary and recouped by the nursing facilities. Please note that the HIDE SNP effective rates illustrated in Table 1 do not include amounts related to the Insurance Provider Assessment (IPA) and Quality Assurance Supplement Payments (QAS), which are included in the CY 2026 capitation rate amounts (final column) that will be paid on a retrospective basis. The estimated amounts for these items are documented in Appendix 3 with discussion of those amounts noted later in this report.

TABLE 1: CY 2026 CAPITATION RATE SUMMARY (PMPM RATES)

RATE CELL	AVERAGE MONTHLY ENROLLMENT	HIDE SNP EFFECTIVE RATE	CY 2026 CAPITATION RATES
Nursing Facility – Subtier A			
Over Age 65	2,475	\$ 7,748.98	\$ 9,338.29
Under Age 65	435	7,542.43	9,080.66
Nursing Facility – Subtier B			
Over Age 65	270	\$ 11,936.13	\$ 13,899.45
Under Age 65	15	11,565.72	13,360.81
Nursing Facility LOC-Waiver			
Over Age 65	2,055	\$ 2,790.78	\$ 2,830.75
Under Age 65	1,380	2,986.78	3,021.34
Community Well			
Over Age 65	41,745	\$ 487.72	\$ 525.53
Under Age 65	25,335	364.80	397.03
Composite	73,710	\$ 886.11	\$ 990.36

Notes:

1. Values shown are on a gross basis prior to reduction for patient pay amounts.
2. Distribution of enrollment based on average projected monthly enrollment for CY 2026.

The projected CY 2026 enrollment estimates were developed based off preliminary January 2026 enrollment information for the MICH program.

FISCAL IMPACT ESTIMATE

Table 2 provides the development of estimated total expenditures, as well as federal only expenditures, under the proposed CY 2026 capitation rates illustrated in Table 1 and projected CY 2026 enrollment. The federal expenditures illustrated in Table 2 are based on the Federal Fiscal Year (FFY) 2026 FMAPs of 65.30%.

TABLE 2: CY 2026 AGGREGATE CAPITATION RATE SUMMARY (\$ MILLIONS)

POPULATION	AGGREGATE HIDE SNP EFFECTIVE EXPENDITURES	AGGREGATE CY 2026 CAPITATION EXPENDITURES
Nursing Facility-Subtier A	\$ 269.5	\$ 324.7
Nursing Facility-Subtier B	40.8	47.4
NFLOC – Waiver	118.3	119.8
Community Well	355.2	384.0
Composite	\$ 783.8	\$ 876.0
Total Federal	511.8	560.5
Total State	272.0	315.5

Notes:

1. Annualized expenditures were developed with projected CY 2026 enrollment.
2. State expenditures based on the Federal Fiscal Years (FFY) 2026 FMAPs of 65.30%.

Section I. Medicaid managed care rates

1. General information

This section provides information listed under the General Information section of CMS guide, Section I.

The capitation rates provided under this certification are “actuarially sound” for purposes of 42 C.F.R. § 438.4(a), according to the following criteria:

- The capitation rates provide for all reasonable, appropriate, and attainable costs that are required under terms of the contract and for the operation of the managed care plan for the time period and population covered under the terms of the contract, and such capitation rates were developed in accordance with the requirements under 42 C.F.R. § 438.4(b).

To ensure compliance with generally accepted actuarial practices and regulatory requirements, we referred to published guidance from the American Academy of Actuaries (AAA), the Actuarial Standards Board (ASB), the Centers for Medicare and Medicaid Services (CMS), and federal regulations. Specifically, the following were referenced during the rate development:

- Actuarial standards of practice applicable to Medicaid managed care rate setting which have been enacted as of the capitation rate certification date, including: ASOP 1 (Introductory Actuarial Standard of Practice); ASOP 5 (Incurred Health and Disability Claims); ASOP 23 (Data Quality); ASOP 25 (Credibility Procedures); ASOP 41 (Actuarial Communications); ASOP 45 (The Use of Health Status Based Risk Adjustment Methodologies); ASOP 49 (Medicaid Managed Care Capitation Rate Development and Certification) ; and ASOP 56 (Modeling).
- Actuarial soundness and rate development requirements in the Medicaid and CHIP Managed Care Final Rules, including but not limited to CMS 2390-F, CMS 2408-F, and CMS 2439-F for the provisions effective for the CY 2026 managed care program rating period.
- The most recent CMS Medicaid Managed Care Rate Development Guide published in August 2025.
- Throughout this document and consistent with the requirements under 42 C.F.R. § 438.4(a), the term “actuarially sound” will be defined as in ASOP 49:

“Medicaid capitation rates are “actuarially sound” if, for business for which the certification is being prepared and for the period covered by the certification, projected capitation rates and other revenue sources provide for all reasonable, appropriate, and attainable costs. For purposes of this definition, other revenue sources include, but are not limited to, expected reinsurance and governmental stop-loss cash flows, governmental risk-adjustment cash flows, and investment income. For purposes of this definition, costs include, but are not limited to, expected health benefits; health benefit settlement expenses; administrative expenses; the cost of capital, and government-mandated assessments, fees, and taxes.”

A. RATE DEVELOPMENT STANDARDS

i. Rate ranges

The capitation rates documented in this report are point estimates and do not represent a rate range.

ii. Annual basis

The actuarial certification contained in this report is effective for the capitation rates for the one-year rate period from January 1, 2026 through December 31, 2026.

iii. Required elements

(a) Actuarial certification

The actuarial certification, signed by Christopher Pettit, FSA, is in Appendix 1. Mr. Pettit meets the qualification standards established by the American Academy of Actuaries, follows the practice standards established by the Actuarial Standards Board, and certifies that the final rates meet the applicable standards in 42 C.F.R. § 438 that are effective for the CY 2026 managed care program rating period.

(b) Certified capitation rates for each rate cell

The certified capitation rates by rate cell are illustrated in Appendix 3. The rates within this report represent the capitation rates prior to application of the area factors, which are additionally illustrated in Appendix 3. For the Nursing Facility rate cells, the rates are developed on a gross basis, prior to the application of patient liability. On an individual basis, MDHHS will deduct the actual patient pay liability amount from the Nursing Facility capitation rate shown in Table 1 and pay the net capitation rate to the HIDE SNPs.

(c) Program information

(i) Managed Care program

MDHHS, along with CMS and the HIDE SNPs, provides benefits for fully dual eligible members under the MICH program within targeted geographic areas. This letter provides the documentation and certification of the calendar year 2026 capitation rates for the Medicaid component of the MICH program.

Beginning in CY 2026, MICH began enrollment among 9 full-risk managed care plans in 4 regions with the anticipation to expand statewide in CY 2027. This certification is for calendar year 2026.

The rate cell structure was developed based upon level of care and age (over/under age 65) with separate area factors applied based on historical experience. The nursing facility rating tier was divided between privately owned (Subtier A) and county owned (Subtier B) facilities.

The services provided under this contract include complete physical and behavioral health, and long-term services and supports. Detailed benefit coverage information for all benefits can be found in the provider agreements.

The program pays secondary to Medicare for Medicare covered services.

Table 3 illustrates the counties included in the MICH program along with their implementation dates.

TABLE 3: MICH REGIONS AND IMPLEMENTATION DATES

MICH REGION	COUNTIES	IMPLEMENTATION DATE
Region 1-Upper Peninsula	Alger, Baraga, Delta, Dickinson, Houghton, Iron, Keweenaw, Luce, Mackinac, Marquette, Ontonagon, and Schoolcraft	January 1, 2026
Region 8-Southwest	Barry, Berrien, Branch, Calhoun, Cass, Kalamazoo, St. Joseph, Van Buren	January 1, 2026
Region 10-Macomb County	Macomb	January 1, 2026
Region 12-Wayne County	Wayne	January 1, 2026

Beneficiaries who reside in a hospice facility are not excluded from the program, however, beneficiaries will not be allowed to enroll from a hospice setting, but rather transition to hospice during enrollment.

(ii) Rating period

This actuarial certification is effective for the one-year rating period January 1, 2026 through December 31, 2026.

(iii) Covered populations

The target population for MICH was limited to full Medicare-Medicaid dual eligible individuals who are age 21 and over and entitled to benefits under Medicare Parts A, B, and D. The program is offered only in select counties across the State of Michigan. These counties include those in the Upper Peninsula, Southwestern Michigan, Macomb county, and Wayne county.

Excluded Populations

The following populations are not eligible for the MICH program and will be excluded from enrollment:

- Individuals under the age of 21
- Individuals previously disenrolled due to special disenrollment from Medicaid managed care as defined in 42 C.F.R. § 438.56
- Individuals not living in a HIDE SNP Service Area
- Individuals who are eligible for partial Medicaid coverage through the following eligibility groups: Additional Low Income Medicare Beneficiary (ALMB), Qualified Individual (QI), Qualified Medicare Beneficiary (QMB), Specified Low-Income Medicare Beneficiary (SLMB), or Qualified Disabled and Working Individuals (QDWI)
- Individuals without full Medicaid coverage (including those with spend downs or deductibles)
- Individuals with Medicaid who reside in a State psychiatric hospital
- Individuals with other comprehensive health coverage including commercial HMO coverage
- Individuals who elected hospice services prior to HIDE SNP enrollment
- Individuals who are incarcerated
- Individuals with presumptive eligibility
- Individuals not eligible for Medicaid due to divestment
- Individuals residing in designated State sanctioned Veterans' Homes

Additional details related to the eligible and excluded populations can be found in the contract between MDHHS and the HIDE SNPs.

The following describes each of the distinct populations which correspond directly with the capitation rate cells.

Nursing Facility Population

This population includes individuals residing in a nursing facility who meet the state definition of nursing home level of care and who are not enrolled in a waiver. Milliman identified the population in the capitation rate-setting process by using fields in the MDHHS eligibility data that denote Medicaid individuals as meeting the nursing home level of care criteria and reside in a nursing facility. The capitation rate for this rate cell was developed based on projected gross nursing facility rates.

On an individual basis, MDHHS will deduct the actual patient pay liability amount from the nursing facility capitation rate shown in Table 1 and pay the net capitation rate to the HIDE SNPs. The nursing facility population is divided into subtiers, split by individuals residing in a privately owned (Subtier A) versus a publicly owned (Subtier B) nursing facility. Additionally, members receiving services in a hospital long-term care facility are categorized in Subtier B based on the average cost identified for these beneficiaries.

Nursing Facility Level of Care-Waiver Population

This population includes individuals who meet the state definition of nursing home level of care, but do not reside in a nursing facility. Eligible individuals must not be enrolled in the State's MI Choice program.

Community Residents Population

This population includes all other qualifying individuals who were not previously categorized. This population is comprised of eligible individuals who are neither institutionalized nor participating in a 1915(c) waiver program.

(iv) Eligibility criteria

Enrollment in the MICH program is not mandatory for eligible individuals. The projected enrollment in the MICH program for CY 2026 is provided by rate cell in Appendix 3.

(v) Special contract provisions

This rate certification report contains documentation of the following special contract provisions related to payment included within rate development.

- Withhold arrangements
- Certain delivery system and provider payment initiatives
- Minimum medical loss ratio requirements

Please see Section I, item 4, Special Contract Provisions Related to Payment, for additional detail and documentation.

(vi) Retroactive adjustment to capitation rates

This rate certification report does not include a retroactive adjustment to the CY 2026 capitation rates.

iv. Differences among capitation rates

Any proposed differences among capitation rates according to covered populations are based on valid rate development standards and are not based on the rate of federal financial participation associated with the covered populations.

v. Cross-subsidization of rate cell payment

The capitation rates were developed at the rate cell level and neither cross-subsidize nor are cross-subsidized by payments from any other rate cell.

vi. Effective dates

To the best of our knowledge, the effective dates of changes to the MICH program are consistent with the assumptions used in the development of the certified CY 2026 capitation rates.

vii. Medical loss ratio

Capitation rates were developed in such a way that the HIDE SNPs would reasonably achieve a medical loss ratio, as calculated under 42 C.F.R. § 438.8, of at least 85 percent for the rate year.

The HIDE SNP contract has remittance provisions with a minimum MLR of 85 percent. The terms and conditions are outlined in Section I, item 4.C.ii.(b).

viii. Capitation Rate Ranges

This section is not applicable because a single set of capitation rates by rate cell were developed for the CY 2026 rating periods.

ix. State's responsibility with rate ranges

This section is not applicable because a single set of capitation rates by rate cell were developed for the CY 2026 rating periods.

x. Generally accepted actuarial practices and principles

(a) Reasonable, appropriate, and attainable

In our judgment, all adjustments to the capitation rates, or to any portion of the capitation rates, reflect reasonable, appropriate, and attainable costs, and have been included in the certification.

(b) Outside the rate setting process

There are no adjustments to the rates performed outside the rate setting process.

(c) Final contracted rates

The CY 2026 capitation rates certified in this report represent the rates by rate cell prior to application of the regional factors. The regional factors are illustrated in Table 4 of this report.

xi. Rate certification for effective time periods

This actuarial certification is effective for the one-year rating period January 1, 2026 through December 31, 2026.

xii. Direct and indirect impacts of COVID-19

We considered the direct and indirect impact of COVID-19 public health emergency (PHE) and relative unwinding on the covered population and services in CY 2026. This analysis and impacts are described in Section I, item 1.B.xi.

xiii. Procedures for rate certification and amendment

In general, a new rate certification will be submitted when the rates change. The following exceptions are allowed per §438.7 of CMS 2390-F:

1. A contract amendment that does not affect the rates.
2. An increase or decrease of up to 1.5% in the capitation rate per rate cell or up to 1% within the certified rate range
3. Risk adjustment, under a methodology described in the initial certification, changes the rates paid to the MCOs

In case 1 and 2 listed above, a contract amendment must still be submitted to CMS.

B. APPROPRIATE DOCUMENTATION

i. Certification type

This report is for the certification of capitation rates and not capitation rate ranges.

ii. Documentation of required elements

This report contains appropriate documentation of all elements described in the rate certification, including data used, assumptions made, and methods for analyzing data and developing assumptions and adjustments.

iii. Medical loss ratio documentation

Using the values illustrated in Appendix 3, the simplified medical loss ratio (defined as the base benefit expense plus DCW add-on divided by the effective capitation rate for purposes of this report net patient pay) is 94.8%. This value is above the minimum standard of 85% and is prior to adjustment for healthcare quality improvement expenses as required in the medical loss ratio definition outlined in 42 CFR § 438.8, which would further increase the pricing medical loss ratio. The HIDE SNPs can reasonably achieve a medical loss ratio of at least 85% as required per 42 CFR § 438.4(b)(9).

We considered the historical medical loss ratios, capitation rate changes, and emerging benefit expense trends when developing the CY 2026 capitation rates as required per 452 CFR § 438.5(b)(5).

iv. Assumptions and adjustments

We attest for all assumptions and adjustments underlying the certified capitation rates which will be disclosed in this rate certification. Rate ranges will not be certified but may be used in developing assumptions and adjustments. The final certified rates reflect specific point estimates.

v. Capitation Rate Ranges

This section is not applicable because a single set of capitation rates by rate cell were developed for the CY 2026 rating period.

vi. Index

The index to this rate certification is the table of contents, found immediately after the title page. The index includes section numbers and related page numbers. Sections not relevant to this certification continue to be provided, with an explanation of why they are not applicable.

vii. Compliance with 42 C.F.R. § 438.4(b)(1)

The capitation rates were developed in a manner consistent with 42 CFR 438.4(b)(1), including that any differences in the assumptions, methodologies, or factors used to develop capitation rates for covered populations are based on valid rate development standards that represent actual cost differences in providing covered services to the covered populations, and that these differences do not vary with the rate of FFP associated with the covered populations in a manner that increases federal costs.

viii. Different FMAP

All populations receive the regular state FFY 2026 FMAP of 65.30% for nine months and the regular FFY 2027 FMAPs for the remaining three months.

ix. Comparison to final certified rates in the previous rate certification.

There is not a previous rate certification for the MICH program to be implemented January 1, 2026 and no prior rates are available for comparison.

x. Known amendments

We anticipate the capitation rates will be amended to reflect future program changes.

These future program changes may include nursing facility per diem updates effective in CY 2026, an adjustment for supplemental benefits funded by Medicare that overlap with Medicaid covered services, and other future program changes not known as of the date of this certification.

The current certification cannot account for the anticipated program changes at this time for the following reasons:

- The nursing facility per diem updates and future program changes are not yet known; and
- Additional collaboration with MDHHS and the HIDE SNPs is needed to further evaluate the overlap in Medicaid covered services from each HIDE SNP's supplemental benefits.

As part of this evaluation, a budget neutral risk adjustment process will be considered to reflect variation of supplemental benefit coverage overlap and utilization of HCBS services by HIDE SNP.

xi. COVID-19 Approach

(a) Data used

For the base data summaries, calendar year 2024 MI Health Link (the state of Michigan's Dual Demonstration program) encounter experience was utilized and summarized in Appendix 2. We did not make any specific adjustments for the PHE caused by COVID-19 outside of the DCW wage adjustment that is continuing into CY 2026. The CY 2024 data represents the most appropriate source of information for rate development when considering the impact the PHE may have had on the MICH program.

(b) Direct and indirect impacts of COVID-19

We did not incorporate an acuity adjustment related to COVID-19 as it was assumed the base experience period reasonably reflects the costs related to COVID-19 in relation to the rating period.

We acknowledge the unwinding of the public health emergency related to COVID-19 that occurred between the base period and the rating period.

(c) Non-risk arrangements

Effective for the CY 2026 contract year, MICH has not implemented any non-risk arrangements related to the COVID-19 PHE.

(d) Risk mitigations strategies used

No additional risk mitigation strategies have been incorporated into the program as a result of the COVID-19 PHE.

2. Data

This section provides information on the data used to develop the capitation rates. The base experience data described in this section is illustrated in Appendix 2.

A. RATE DEVELOPMENT STANDARDS

In accordance with 42 C.F.R. §438.5(c), we have followed the rate development standards related to base data. The remainder of Section I, item 2 provides documentation of the data types, sources, validation process, material adjustments and other information relevant to the documentation standards required by CMS.

B. APPROPRIATE DOCUMENTATION

i. Requested data

As the actuary contracted by MDHHS to provide consulting services and associated financial analyses for many aspects of the Michigan Medicaid program (and not just limited to capitation rate development), Milliman intakes and summarizes eligibility and expenditure data on a monthly basis from Optum, MDHHS's data administrator. We received eligibility and expenditure information historical time periods. There was no data that was requested from Milliman that was not received. The remainder of this section details the base data and validation processes utilized in the CY 2026 capitation rate development.

ii. Data used to develop the capitation rates

(a) Description of the data

(i) Types of data

The CY 2024 encounter data for the prior MI Health Link program served as the primary data source for the CY 2026 capitation rate development for the MICH program. While fee-for-service (FFS) data for other Michigan Medicaid programs were available to us, the managed care encounter data from the prior MI Health Link program was selected to serve as the base data due to its consistency to the MICH program with regard to covered services, populations, managed care efficiencies, and other program-specific contractual requirements. The FFS data were used in the development of new population acuity adjustments as discussed in Section I, item 2.B.iii.d.

The following data sources were utilized to inform adjustments to the plan-submitted encounter data:

- Encounter data for the MI Health Link population
- Managed care enrollment data
- Survey submissions completed by each HIDE SNP
- Additional gross adjustment expenditure information outside the MMIS claims system
- Summary of known policy and program changes (including changes to fee schedules and other payment rates)
- Information reported from DCW providers from the CY 2025 MHL DCW Provider Survey

Appendix 2 illustrates the encounter base data summaries that provide the foundation for the calendar year 2026 MICH capitation rate development. The information is stratified by rate cell and category of service.

(ii) Age of the data

The data serving as the base experience in the capitation rate development process was incurred during CY 2024 and reflects encounters accepted through July 2025.

For the purposes of trend development and analyzing historical experience, we also reviewed encounter and enrollment experience through September 2025. We utilized recent average monthly enrollment for purposes of emerging population enrollment patterns.

(iii) Data sources

The historical claims and enrollment experience for the data obtained through the warehousing process was provided to Milliman by Optum, the data administrator for MDHHS. The sources of other data are noted in (i) and (ii) above.

(iv) Sub-capitation

There were no sub-capitated claims identified in the historical encounter data for the MICH program.

(v) Exceptions

No exception to base data requirements has been requested by the state or granted by CMS due to the COVID-19 public health emergency.

(b) Availability and quality of the data**(i) Steps taken to validate the data**

The base experience used in the capitation rates relies on encounter data submitted to MDHHS by ICOs from the MI Health Link program. Managed care eligibility is maintained in the data warehouse by MDHHS. The actuary, the ICOs, and MDHHS all play a role in validating the quality of encounter data used in the development of the capitation rates. The ICOs play the initial role, collecting and summarizing data sent to the state. MDHHS works with the data warehouse managers on data quality and ICO performance measurement. Additionally, we perform independent analysis of encounter data to evaluate the quality of the data being used in the rate development process. Below is a summary of measures specific to each quality area that are applied by MDHHS or the actuary. We anticipate utilizing HIDE SNP data in future rate development processes as emerging MICH experience becomes available.

Completeness

As the actuary, we summarize the encounter data to assess month to month completeness of the encounter data. These measures include:

- Encounter per member per month (PMPM) cost for high level service categories;
- Distribution of members by encounter-reported expenditures; and,
- Review of month-to-month activity across the program and rate cell.

These measures are applied to identify any months where encounter data volume is unusually large or small, indicating a potential issue with the submitted encounter data.

The CY 2024 encounter data used in the development of the rates includes claims adjudicated and submitted to MDHHS through July 2025. Minor completion factors were applied to the CY 2024 base data for estimated claims paid after July 2025.

Encounter data is summarized through the encounter quality initiative (EQI) process. Separate summaries, by rate group, are prepared for each ICO. Each summary illustrates utilization, cost per service, and per member per month cost for the population, stratified by category of service. The format of each exhibit is similar to the base data exhibits that are provided in Appendix 2, allowing most data issues to be discovered before the annual capitation rate development process.

The EQI reconciliation process allows for months of run-out from the end of the reported evaluation period. The actuary compares the EQI summaries to summary totals submitted by the ICOs.

We provide all the individual encounter claims back to the ICOs for analysis. This allows the ICOs to identify any claims that need to be resubmitted or research any discrepancies that may exist in the final summary.

Accuracy

Checks for accuracy of the data begin with the ICOs' internal auditing and review processes. MDHHS reviews the accuracy of the encounter data by reviewing the percentage of accepted encounters between the MDHHS encounter data files and the files submitted by the health plans.

As the state actuary we also review the encounter data to ensure each claim is related to a covered individual and a covered service. Claims utilized in the rate development process are those that have matching beneficiary IDs that are eligible for the noted service date.

We summarize the encounter data into an actuarial cost model format. Base period data summaries are created to ensure that the data for each service is consistent across the health plans and with prior historical periods. Stratification by rate cell facilitates this review, as it minimizes the impact of changes in population mix. This process identifies health plan and service category combinations that may have unreasonable reported data.

Consistency of data across data sources

As historical encounter data is the primary source of information used in the development of capitation rates effective January 1, 2026, it is important to assess the consistency of the encounter data with other sources of information. The main sources of comparison were the ICO-submitted EQI reports, in which each ICO submitted exposure and expenditure information that covered the time periods starting from January 2024. We utilized the EQI reported data to validate the encounter data being utilized for rate development was appropriate and consistent between the two sources of information.

We also reviewed the consistency of the monthly enrollment in eligibility and capitation payment files received by MDHHS was reconciled with publicly available values.

(ii) Actuary's assessment

As required by Actuarial Standard of Practice (ASOP) No. 23, Data Quality, we disclose that Milliman has relied upon certain data and information provided by MDHHS and their vendors, primarily the ICOs. The values presented in this letter are dependent upon this reliance.

We found the encounter to be of appropriate quality for purposes of developing actuarially sound capitation rates. The following actions were performed to ensure compliance with ASOP 23:

- Selected data that were both appropriate and sufficiently current for the intended purpose: we used data that reflected the covered population and services under the contract;
- Reviewed the data for reasonability, consistency, and comprehensiveness: documented in the certification report;
- Disclosed any known limitations of the data: documented in the certification report; and,
- Placed reliance on the data supplied by MDHHS and its vendors: documented in the certification report.

While there are areas for data improvement, we found the encounter data to be of appropriate quality for the purposes of developing the base experience data for the capitation rates, as well as specific adjustments for reimbursement and program changes that impact expenditures beyond the base experience period.

(iii) Data concerns

We have not identified any material concerns with the quality or availability of the data specific to CY 2024. Based on a review of encounter data at the population and category of service level for each ICO, we deemed the encounter data experience to be of sufficient quality. This analysis was done on a consistent basis across each ICO.

Additionally, we reviewed the historical experience to identify duplicate claims in the base experience. We do not believe these concerns are material enough to exclude them from the encounter data. We believe the encounter data using in our rate development process is reasonable and credible.

(c) Appropriate data

(i) Use of encounter and fee-for-service data

All populations enrolled in managed care during the rate period were included in the risk-based managed care delivery system in the CY 2024 base experience period. As such, expenditure data for populations enrolled in FFS during CY 2024 is not reflected in the base experience cost models used to develop the capitation rates.

(ii) Use of managed care encounter data

Managed care encounter data was the primary data source used in the development of the capitation rates.

(d) Reliance on a data book

Development of the capitation rates did not rely on a data book or other summarized data source. We were provided with detailed claims data for all covered services and populations.

iii. Data adjustments

Capitation rates were developed from historical calendar year 2023 encounter data, paid through July 2025. As shown in Appendix 2, the primary base data year adjustments include completion, trend, reimbursement, and other program adjustments.

(a) Credibility adjustment

The MICH eligible populations, in aggregate, were considered fully credible.

As described in the previous data validation section, the encounter data is compared to the EQI health plan reported information. Minor adjustments to the base experience were incorporated to better align the base data with the plan reported amounts.

(b) Completion adjustment

The capitation rates are based on calendar year 2024 experience. Encounter data reflects claims paid and submitted to MDHHS through July 2025 and includes seven months of claims run-out. Based on a review of historical completion patterns for claims in this program, minor completion adjustments were applied to the base experience period data.

An additional adjustment was applied to adjust for a material difference observed in the Other Inpatient service category in the base data compared to the emerging experience. This adjustment reflects a decrease of approximately \$10.4 million dollars and brings the composite PMPM of inpatient services in line with observed levels for most ICOs and the projected FFS members rolling into the MICH program.

(c) Errors found in the data

As previously noted, we reviewed claims that were identified as duplicates. Potential duplicate claims were identified based on service date, beneficiary ID, applicable service codes (e.g., CPT-4 procedure code), and paid amounts. We did not identify a material amount of duplicate claims, so no adjustment to the base data was made.

(d) Program change adjustments

The base data year represents a historical time period from which projections were developed. We reviewed materials from MDHHS to identify program changes that were implemented during the base data period. To the extent the program adjustments were estimated to have a material impact on MICH service costs an adjustment was considered for the calendar year 2026 rate development process. Adjustments were made to the portion of the base data prior to the implementation of each program change in order to ensure the entire base period was on a consistent basis.

Dental Adjustment

Effective April 1, 2023, MDHHS implemented policy changes to redesign the adult dental services. These changes included increased reimbursement rates for providers, improved access and provider participation, reduced administrative burdens for providers, and expanding the benefit package to ensure robust coverage across Medicaid-covered populations. Illustrated in appendix 2 by rate cell, the CY 2026 capitation rates have been adjusted to include an additional \$2.3 million to fund the cost of providing coverage for these expanded services and increased reimbursement rate.

Minimum Wage and Earned Sick Time

Effective February 21, 2025, Michigan's Earned Sick Time Act (ESTA) went into effect and the minimum wage was increased from \$10.33 to \$12.48 per hour.

Additionally, Michigan is planning to increase the minimum wage further to \$13.73 per hour effective January 1, 2026. Both the minimum wage increases and the implementation of ESTA are anticipated to increase costs to provide HCBS services by direct care workers. We have adjusted the CY 2026 capitation rates by \$22.5M to account for the increased costs associated with these changes. This adjustment was developed based on review of surveys issued directly to direct care worker providers. The surveys collected information about increased wages and changes in paid time off because of these policy changes specific to the MICH Program. This adjustment represents increases for all providers listed in the survey, not solely providers earning below the new minimum wage. Appendix 3 documents the PMPM impact by rate cell.

HCBS Acuity Adjustment

Effective January 1, 2026, projected enrollment in the MICH program is anticipated to be materially higher than the prior MI Health Link managed care program, with the increase in dual eligible members coming from prior FFS Medicaid coverage. Reviewing the historical experience, we note a material difference in utilization of HCBS services, most predominately observed for the community well population. We have included a reduction to the projected utilization of HCBS services compared to the ICO encounter base data to take into consideration the lower acuity of services needed for the historical FFS population. This adjustment represents a decrease of approximately \$9.3 million and is illustrated in Appendix 3 by rate cell for the HCBS service category. In addition to differences in utilization, we note that the FFS experience represented an average cost per service materially lower than the ICO encounter data. As part of this adjustment, we have only reflected the utilization difference, with the assumption that reimbursement for these services for the prior FFS members will be consistent with the ICO level once covered under the MICH program.

Medicare Advantage Maximum Out-of-Pocket Limit

In CY 2026, each enrollee's maximum out of pocket (MOOP) limit for Medicare Advantage is \$9,250. Once an enrollee reaches the Medicare out-of-pocket limit, we assume the Medicare Advantage plan will be liable for all further cost-sharing of Medicare covered services. We summarized expenditures in the base period that would not be subject to Medicaid cost sharing in the base experience for each individual that reaches the MOOP limit. We developed an adjustment to remove the excess funds allocated to Medicaid that would occur following the MOOP limit. This adjustment represents a decrease of approximately \$2.5 million and is illustrated in Appendix 3 by rate cell and service category.

Supplemental Benefits Overlap Adjustment

We understand that a HIDE SNP may not be paid twice for covering the identical benefit to the same enrollees in the same contract year. We reviewed the Plan Benefit Packages (PBPs) submitted to CMS and underlying details for the rating period to identify supplemental benefits otherwise covered by Medicaid. Benefits identified as having a material overlap consisting of comprehensive dental, non-emergency transportation, home modifications, and personal emergency response systems. We quantified the Medicaid costs associated with those benefits and removed them from the projected Medicaid benefit expenses. Note that additional supplemental benefit coverage was reviewed and taken into consideration as part of the overall rate development process (i.e., trend selection) but not included as part of the explicit adjustment. Adjustment factors were developed at the rate cell and service category level considering each HIDE SNPs supplemental benefit offerings and the projected enrollment. In composite, this adjustment represents a projected decrease of approximately \$16.0 million and is illustrated in Appendix 3 by rate cell and service category.

DCW Premium Pay

Reflected in the CY 2024 base data, expenses reported for DCW services included the increased hourly rates of \$3.60 per hour for premium pay in response to the COVID-19 pandemic. Effective for the CY 2026 rating period, this adjustment was increased to \$3.84 per hour, which includes additional allocation to cover employer related costs consideration for indirect time and overtime. Appendix 2 documents the initial adjustment to remove amounts reflected in the base data. Appendix 3 documents the subsequent adjustment to add funding back in at the higher hourly rate. The total amount of funding included in the capitation rates for the DCW wage increase for the CY 2026 rating period is approximately \$64.3 million.

(e) Exclusion of payments or services from the data

No specific payments were excluded from the rate development.

3. Projected benefit cost and trends

This section provides information on the development of projected benefit costs in the capitation rates.

A. RATE DEVELOPMENT STANDARDS

i. Final Capitation Rate Compliance

The final capitation rates are in compliance with 42 C.F.R § 438.4(b)(6) and are only based on services outlined in 42 C.F.R § 438.3(c)(1)(ii) and 438.3(e). The HIDE SNPs do not provide any in-lieu-of services.

ii. Benefit Cost Trend Assumptions

Projected benefit cost trend assumptions are developed in accordance with generally accepted actuarial principles and practices. The primary data used to develop benefit cost trends is historical claims and enrollment from the covered populations. Additionally, consideration of other factors and data sources appropriate for benefit cost trend development is further documented in Section I, item 3.B.iii.

iii. In Lieu Of Services

The projected benefit costs do not include costs for in lieu of services.

iv. IMDs as an in lieu of service provider

Not applicable. The projected benefit costs do not include costs for in lieu of services.

(a) Costs associated with an IMD stay of more than 15 days

We reviewed benefit costs for enrollees aged 21 to 64 during the base experience period and determined that projected benefit costs do not include costs associated with an Institution for Mental Diseases (IMD) stay of more than 15 days in a month. Therefore, we have not included an adjustment to the base experience data for IMD and associated expenses.

(b) Other costs for services during the time an enrollee is in an IMD for more than 15 days

We have not included any costs in the base experience data for associated expenses when a member is in an IMD for more than 15 days.

B. APPROPRIATE DOCUMENTATION

i. Projected Benefit Costs

This section provides the documentation of the methodology utilized to develop the benefit cost component of the capitation rates at the rate cell level.

ii. Development of Projected Benefit Costs

(a) Description of the data, assumptions, and methodologies

This section of the report outlines the data, assumptions, and methodology used to project the benefit costs to the rating period. The baseline benefit costs were developed using the following steps:

Step 1: Create unadjusted cost model summaries

The capitation rates were developed from historical encounter and enrollment data for members enrolled in the MI Health Link program for the noted demonstration regions. This data consisted of calendar year 2024 incurred experience that was submitted by the ICOs to the data warehouse and maintained by Optum. This information serves as the starting point of the base experience and is noted as unadjusted CY 2024 experience in Appendix 2.

Step 2: Adjust for retrospective base data adjustment, prospective program and policy changes, and trend to calendar year 2026

The base experience period was adjusted for the items outlined in Section I, item 2. We applied a completion adjustment and adjustments to reflect policy and program changes that occurred during CY 2024. For retrospective program adjustment factors, the adjustments were developed at either the population level or at the rate cell level, as appropriate.

Adjustment factors were developed and applied to the CY 2024 base experience to reflect known policy and program changes that have occurred or are anticipated to be implemented from January 1, 2024 through December 31, 2026.

Full documentation of currently known items that were considered in the rate development is provided in this report. The adjusted PMPM values from the base experience period were trended forward to the midpoint of the contract period (July 1, 2026). We have included one-time specific adjustments to the Inpatient Hospital (approximately 5.0% increase) and Professional (approximately 0.40% increase) cost trend adjustments to account for the changes in Medicaid cost sharing due to the increases in Medicare Part A and Part B deductibles between the base experience period and CY 2026. Further, we have included additional cost per service adjustment for the Nursing Facility services to align the projected reimbursement level with the fee schedule per diem rate. The adjustments are reflected on Appendix 2 in the columns noted as Cost Trend Adjustment in combination with the applied annualized trend. These explicit adjustments have increased the effective trend to be above the selected trend assumptions identified in Appendix 4.

Adjustments were applied to the PMPM values to reflect program changes between the base period and effective rate period. The resulting PMPMs establish the adjusted claim cost by population for the contract period. Consistent with step 2, program adjustment and trend factors were developed at either the population level or at the rate cell level, as appropriate.

Step 3: Adjust for additional payments and reimbursement changes

Additional adjustments were applied to the base experience to reflect the impact of the following:

- **Supplemental SNF Copayments** - The supplemental nursing facility patient pay amount is reflected on a per member per month (PMPM) basis for members in a nursing facility rate tier in Appendix 3.
- **Direct-Care Worker (DCW) premium pay adjustment** MDHHS increased reimbursement for DCW services to \$3.84 per hour effective for the CY 2026 rating period. Appendix 3 documents the adjustment made to underlying base experience for the increased reimbursement amounts for DCW services.

Step 4: Incorporate non-benefit items and adjustments

The CY 2026 capitation rates include an allowance for non-benefit cost items. This includes administrative allowance, Insurance Provider Assessment (IPA) tax, Quality Assurance Supplemental (QAS), and provision for underwriting margin. The provision for margin includes financing related to the cost of capital and contingency against general losses.

Step 5: Regional adjustments

The rates noted in Table 1 represent the statewide rate for each rate cell. Capitation rates paid to each of the HIDE SNPs will be dependent upon the region for which the covered life resides. Consistent with the four regions identified in Table 3, regional adjustment factors were calculated for each applicable region and rating tier.

The relative cost per service differences for key service lines was utilized to develop the regional adjustment factors applied to the capitation rates.

For the nursing facility populations, the regional adjustment factor was developed by comparing the nursing home per diem rates specific to each region to the composite per diem rate across all regions for each population. The community well regional factor was based on comparing the average cost per member per month of personal care services by each region. The variation in average cost for these services drives a significant portion of the difference in overall expenditures by region. The regional factors adjust the composite rate in line with the expected cost variation as a result of these differences for each rating tier and region. We have not identified the need to regionally adjust rates for the Subtier B or Waiver tiers and have noted these as 1.0 in the table below.

The following table documents the developed regional adjustment factors by demonstration region and population. Separate regional adjustments were not developed for Over/Under 65 rate cells.

TABLE 4: REGIONAL ADJUSTMENT FACTORS (ON HIDE SNP EFFECTIVE RATE)

MICH REGION	NURSING FACILITY - SUBTIER A	NURSING FACILITY - SUBTIER B	WAIVER (NF LEVEL OF CARE)	COMMUNITY WELL
Region 1	0.9530	1.0000	1.0000	0.8895
Region 8	1.0178	1.0000	1.0000	0.9603
Region 10	1.0157	1.0000	1.0000	1.1231
Region 12	1.0009	1.0000	1.0000	0.9970

(b) Material changes to the data, assumptions, and methodologies

The primary change from the prior year rate-setting is re-basing the capitation rate development with CY 2024 base experience. All material assumptions are documented in this rate certification report and the overall methodology utilized to develop the capitation rates is consistent with the prior rate-setting analysis.

(c) Overpayments to providers

Consistent with 42 C.F.R. § 438.608(d), MDHHS outlines the program integrity guidelines and reporting requirements related to overpayments, recoveries, and refunds in the HIDE SNP contract.

iii. Projected Benefit Cost Trends

This section discusses the data, assumptions, and methodologies used to develop the benefit cost trends, i.e., the annualized projected change in benefit costs from the historical base period (CY 2024) to the CY 2026 rating period of this certification. We evaluated prospective trend rates using historical experience for the Michigan Medicaid managed care program, as well as external data sources.

(a) Required elements**(i) Data**

The primary source of data used in the development of historical trends was encounter data through September 2025 specific to the prior MI Health Link program.

External data sources that were referenced for evaluating trend rates developed from MDHHS data include:

- *National Health Expenditure (NHE) projections* developed by the CMS office of the actuary, specifically those related to Medicaid. Please note that as these are expenditure projections, projected growth reflects not only unit cost and utilization, but also aggregate enrollment growth and enrollment mix changes such as aging.

For trends used in this certification, we are interested only in unit cost and utilization trends, so in general, our combinations of unit cost and utilization trends should be lower than NHE trends. NHE tables and documentation may be found in the location listed below:

- <https://www.cms.gov/research-statistics-data-and-systems/statistics-trends-and-reports/nationalhealthexpenddata/nationalhealthaccountsprojected.html>

Other sources: We also reviewed internal sources that are not publicly available, such as historical experience from other programs and trends used by other Milliman actuaries.

(ii) Methodology

For internal MDHHS data, historical utilization and PMPM cost data was stratified by month, rate cell, and major category of service. The data was adjusted for completion and normalized for historical program and reimbursement changes. We used linear regression to project experience during the contract period. Contract period projections were compared to base period experience to determine an appropriate annualized trend.

(iii) Comparisons

Historical trends should not be used in a simple formulaic manner to determine future trends; a great deal of actuarial judgment is also needed. We did not explicitly rely on the historical encounter data trend projections. We referred to the sources listed in the prior section, the impact of reimbursement changes on utilization, and shifting population mix.

(iv) Chosen trend rates

Appendix 4 provides the selected trend rates by category of service. These trends include both utilization and cost per service components. As noted above, the cost trend adjustment reflected in Appendix 2 is higher than the selected cost trend to align with the projected reimbursement level based on the Nursing Facility fee schedule per diem rate.

(b) Benefit cost trend components

Separate utilization and cost per unit trend components were developed and illustrated in Appendix 4 along with the results of the regression analyses performed to evaluate the historical trend experience.

(c) Variation

We developed trends by major category of service. Trend variations between service categories reflect observed variation in the underlying historical experience and actuarial judgement based on the sources identified in the section above.

(i) Medicaid populations

To limit the variation in benefit cost that is present across the Medicaid population, we developed trends by major category of service. Minor trend variations between populations and service categories reflect observed variation in the underlying historical experience and actuarial judgement based on the sources listed in the section above.

(ii) Rate cells

Benefit cost trends are evaluated by major category of service. For population categories comprised of multiple rate cells, the benefit cost trends are consistent across all rate cells.

(iii) Subsets of benefits within a category of services

We did not vary trend assumption within a category of service.

(d) Material adjustments

Historical trends should not be used in a simple formulaic manner to determine future trends; a great deal of actuarial judgment is also needed. We did not solely rely on the historical encounter data trend projections due to anomalies observed in the historical trend data. We referred to the sources listed in the prior section, considered changing practice patterns, the impact of reimbursement changes on utilization in the populations, and shifting population mix.

We made adjustments to trend rates derived from historical experience in cases where the resulting trends did not appear reasonably sustainable or were not within consensus parameters derived from other sources. For many rate cells and categories of services, raw model output was outside of a range of reasonable results. In these situations, we relied on the methodologies identified to develop prospective trend.

(e) Any other adjustments**(i) Impact of managed care**

We did not adjust the trend rates to reflect a managed care impact on utilization or unit cost.

(ii) Trend changes other than utilization and cost

We did not adjust the benefit cost trend for changes other than utilization or unit cost.

iv. Mental Health Parity and Addiction Equity Act Service Adjustment

We have reviewed MDHHS's state plan benefits regarding compliance with the Mental Health Parity and Addiction Equity Act of 2008 (MHPAEA) as required by 42 C.F.R. § 438.3(c)(1)(ii). Results of the analysis indicate full compliance with MHPAEA for financial requirements and both quantitative and non-quantitative treatment limits with no remediation needed that would constitute a program adjustment. Based on the final results, we have not made any rating adjustments to accommodate parity compliance.

v. In Lieu of Services

The projected benefit costs do not include costs for in lieu of services.

vi. Retrospective Eligibility Periods

(a) HIDE SNP responsibility

HIDE SNPs are not responsible for paying claims incurred during the retrospective eligibility period.

(b) Claims treatment

As noted earlier, HIDE SNPs are not responsible for paying claims incurred during the retrospective eligibility period; therefore, claims for retrospective eligibility periods are not reflected in the base data.

(c) Enrollment treatment

Enrollment is treated consistently with claims. We have not included retrospective eligibility in the base experience period.

(d) Adjustments

No adjustments are necessary.

vii. Impact of Material Changes

This section relates to material changes to covered benefits or services since the last rate certification. There is not a previous rate certification for the MICH program to be implemented January 1, 2026 and no prior certification is available for comparison. Descriptions in this section refer to changes from the base period.

(a) Change to covered benefits

Material changes to covered benefits have been described in program adjustments described in Section I, item 2.B.iii Program Change Adjustments.

(b) Recoveries of overpayments

To the best of our knowledge, all information related to any payment recoveries is reflected in the base period data.

(c) Change to payment requirements

Material changes to required provider payments have been described in program adjustments described in Section I, item 2.B.iii Program Change Adjustments.

(d) Change to waiver requirements

There were no material changes related to waiver requirements or conditions.

(e) Change due to litigation

There were no material changes due to litigation.

viii. Documentation of Material Changes

Material changes to covered benefits and provider payments have been described in program adjustments described in Section I, item 2.B.iii Program Change Adjustments. This information includes the data, assumptions, and methodology used in developing the adjustment, and aggregate impact on the managed care program's benefit expense.

4. Special Contract Provisions Related to Payment

A. INCENTIVE ARRANGEMENTS

i. Rate Development Standards

This section provides documentation of the incentive payment structure in the MICH program.

ii. Appropriate Documentation

There are currently no explicit incentives in the HIDE SNP contracts.

B. WITHHOLD ARRANGEMENTS

i. Rate Development Standards

There is currently no withhold arrangement in the MICH program for the CY 2026 contract period.

C. RISK SHARING MECHANISMS

i. Rate Development Standards

This section provides documentation of the risk-sharing mechanisms in the MICH managed care program.

ii. Appropriate Documentation

(a) Description of Risk-sharing Mechanism

No risk sharing arrangements exist for the covered populations outside of the minimum medical loss ratio.

(b) Medical Loss Ratio

Description

Effective CY 2026, each HIDE SNP is required to meet a minimum medical loss ratio (MLR) threshold which regulates the minimum amount (as a percentage of the Medicaid payments) that must be used for expenses either directly related to medical claims or those which are related to the care and quality of enrollees. This was established at 85%.

Financial consequences

If a HIDE SNP has an MLR below the target of 85%, the HIDE SNP will remit the difference between its MLR and 85%.

(c) Reinsurance Requirements and Effect on Capitation Rates

The standard contract language between the state and the HIDE SNPs requires contractors to maintain certain insurances as identified in the contract terms. These would include general liability insurance. Contractors are also required to utilize subcontractors with similar insurance coverages.

D. STATE DIRECTED PAYMENTS

i. Rate Development Standards

Consistent with guidance in 42 C.F.R. §438.6(c), the Michigan managed care capitation rates reflect consideration of the following delivery system and provider payment initiatives (i.e., state directed payments):

- Uniform dollar increase for nursing facilities (QAS program).
- HCBS Direct Care Worker premium pay (DCW)

(a) Description of Managed Care Plan Requirement

QAS: The add-on QAS payment amounts were originally developed based on the fee-for-service (FFS) model approved in the state plan. In order to maintain this portion of the nursing facility daily rate it is being transitioned to a directed payment.

Michigan also performs an annual calculation of the Medicare UPL for nursing facilities. The total payments for these services remain below the UPL. As such, the QAS payments are reasonable, equitable, and appropriate for these providers.

DCW: MDHHS requires the HIDE SNPs to participate in a state directed initiative to reimburse direct care workers at a higher hourly rate to ensure access to Medicaid beneficiaries.

(b) Written Approval

MDHHS has submitted and awaiting written approval for the QAS state directed payment delivery systems with the pre-print control name of MI_Fee_NF_Renewal_20260101-20261231.

MDHHS is awaiting written approval for the DCW state directed payment delivery systems for CY 2026 with the pre-print control name of MI_Fee_HCBS2_Renewal_20260101-20261231.

(c) Actuarial Standards

Payments for the QAS and DCW delivery systems were developed in accordance with 42 C.F.R. § 438.4, the standards specified in § 438.5, and generally accepted actuarial principles and practices.

(d) How Payment Arrangement is Reflected in Managed Care Rates

The QAS payments within the various directed payment initiatives are done so on a retrospective basis to the HIDE SNPs. The DCW payments are included in the development of the monthly base capitation rates paid to the managed care health plans.

(i) Documentation related to payment term included in the rate certification

Documentation related to the separate payment term is addressed in Section I, Item 4.D.ii(a)(iii).

(ii) PMPM estimate of state-directed payments addressed through separate payment term

PMPM estimates related to the state directed payments are included in Appendix 3 of this certification report.

(iii) Final documentation of total state-directed payment amount by rate cell

To the extent the final state-directed PMPM payments by rate cell vary from the initial estimates an amendment will be developed including a certification of the final capitation rates.

(iv) Change from initial base rate certification

As indicated above, the rate certification will be updated through a rate amendment if the total payment amount or distribution methodology varies from the initial estimates.

ii. Appropriate Documentation**(a) Description of Delivery System and Provider Payment Initiatives****(i) Description of delivery system and provider payment Initiatives included in the capitation rates**

Utilization of the following delivery system and provider payment initiatives will be included in the final capitation rates:

CONTROL NAME	TYPE OF PAYMENT	BRIEF DESCRIPTION	RATE ADJUSTMENT OR SEPARATE PAYMENT TERM
MI_Fee_NF_Renewal_20260101-20261231 (pending approval)	Uniform dollar increase	See description below.	Separate payment term
MI_Fee_HCBS2_Renewal_20260101-20261231 (pending approval)	Uniform dollar increase	See description below.	Rate Adjustment

- **QAS.** Uniform dollar increase for nursing facilities for the rating period covering January 1, 2026 through December 31, 2026
- **Direct care worker premium program.** The Direct Care Worker (DCW) program was developed to ensure continued access by Medicaid beneficiaries to home and community-based services.

(ii) Description of payment arrangement if incorporated as a rate adjustment

The DCW payment arrangements will be incorporated through a rate adjustment applied in the development of the monthly base capitation rates paid to plans.

CONTROL NAME	RATE CELLS AFFECTED	AGGREGATE AMOUNT	PMPM MAGNITUDE	PREPRINT CONSISTENCY CONFIRMATION
MI_Fee_HCBS2_Renewal_20260101-20261231 (pending approval)	All rate cells	\$64.3 million	Listed by rate cell in Appendix 3	This state directed payment is consistent with the preprint.

(iii) Description of payment arrangement if incorporated as a separate payment term

The QAS payment arrangements will be incorporated through a separate payment term in which the monthly capitation rate will be directed to the eligible nursing facilities based on actual utilization.

CONTROL NAME	AGGREGATE AMOUNT	PMPM MAGNITUDE	PREPRINT CONSISTENCY CONFIRMATION	SUBMIT REQUIRED DOCUMENTATION REQUIREMENT
MI_Fee_NF_Renewal_20260101-20261231	\$65.2 million	Listed by rate cell in Appendix 3	This state directed payment is consistent with the preprint.	MDHHS will provide required documentation following CY 2026 with actual amounts paid on a retrospective basis.

(b) Additional directed payments not addressed

Not applicable.

(c) Requirements regarding reimbursement rates

There are no requirements regarding the reimbursement rates the managed care plan(s) must pay to any providers unless specifically specified in the certification as a state directed payment or authorized under applicable law, regulation, or waiver.

E. PASS-THROUGH PAYMENTS

i. Rate Development Standards

There are no pass-through payments reflected in the CY 2026 capitation rates.

ii. Appropriate Documentation

There are no pass-through payments reflected in the CY 2026 capitation rates.

5. Projected non-benefit costs

A. RATE DEVELOPMENT STANDARDS

i. Overview

In accordance with 42 C.F.R. §438.5(e), the non-benefit component of the capitation rate includes reasonable, appropriate and attainable expenses related to HIDE SNP operation of the MICH program.

The remainder of Section I, item 5 provides documentation of the data, assumptions and methodology that we utilized to develop the non-benefit cost component of the capitation rate.

ii. PMPM versus percentage

The non-benefit cost was developed as a percentage of the capitation rate for general administrative costs and underwriting margin.

An additional component of the non-benefit expense is the insurance provider assessment (IPA) that is applicable to insurance providers in the State of Michigan. The IPA assesses a PMPM rate of \$53.55 to each covered member month, by managed care entity, up to 1.2 million members in a given state fiscal year. The PMPM amount decreases to \$1.20 for each member month above 1.2 million. The ultimate amount paid for the IPA will be vary by managed care entity based on actual enrollment utilized in the calculation of the assessment. We have included a PMPM estimate for CY 2026 in Appendix 3 based on a complete 12 months' worth of payments over the calendar year. Please note that we have developed a singular PMPM across all HIDE SNPs but acknowledge that ultimate amounts paid will vary by HIDE SNPs.

B. APPROPRIATE DOCUMENTATION

i. Development of non-benefit costs

(a) Description of the data, assumptions, and methodologies

Data

The primary data sources used in the development of the calendar year 2026 non-benefit costs are listed below:

- CY 2026 HIDE SNP rate setting surveys.
- Benchmark non-benefit costs for other managed care programs.

Assumptions and methodology

In developing the administrative costs, we reviewed the CY 2026 HIDE SNP rate setting surveys and found a material amount of variation in the reporting of administrative expenses among the HIDE SNPs.

Historical administrative cost allowances in the CY 2026 capitation rates attributable to the prior MI Health Link program were used as the starting point due to the similarities in the administrative requirements with the MICH program. We reallocated the composite administrative load expenses by rate cell based on an analysis of administrative requirements and the underlying benefit expense by rate cell. After the reallocation, we considered annual inflation adjustment to these historical administrative cost allowances when selecting the non-benefit cost allowance assumptions.

(b) Material changes

There is not a previous rate development for the MICH program to be implemented January 1, 2026. As a result, there are no material changes since the last rate certification.

(c) Other material adjustments

There are no other material adjustments applicable to the non-benefit cost component of the capitation rate.

ii. Non-benefit costs, by cost category

Administrative expenses have not been developed from the ground up (based on individual components), but underlying administrative requirements were reviewed when selecting the non-benefit cost allowance.

The CY 2026 non-benefit cost allowance is applied as a percentage of the capitation rates excluding directed payments, IPA, and patient pay, as illustrated in Table 5 below.

TABLE 5: NON-BENEFIT EXPENSE COST AS PERCENTAGE OF EFFECTIVE RATE

NON-BENEFIT EXPENSE	NURSING FACILITY – SUBTIER A	NURSING FACILITY – SUBTIER A	NURSING FACILITY LOC – WAIVER	COMMUNITY WELL
Administrative Expenses	2.50%	2.25%	5.00%	7.00%
Underwriting Margin	1.00%	1.00%	1.00%	1.00%
Total Non-Benefit Expense	3.50%	3.25%	6.00%	8.00%

The resulting value of these items on a PMPM basis can be reviewed by rate cell in Appendix 3.

iii. Historical non-benefit costs

Historical non-benefit costs were reported through HIDE SNP rate survey submissions. We evaluated the historical program costs along with nationwide administrative cost benchmarks and trends when establishing the non-benefit expense amounts. Due to the similarities in administrative requirements between the prior MI Health Link program and the MICH program, consideration of historical non-benefit costs is reasonable and appropriate.

6. Risk Adjustment

This section provides information on the risk adjustment included in the contract.

A. RATE DEVELOPMENT STANDARDS

i. Overview

The MICH rates have been developed as full risk rates. The HIDE SNPs assume risk for the cost of services covered under the contract and incur losses if the cost of furnishing the services exceeds the payments under the contract. There is no risk adjustment arrangement established for the CY 2026 contract period.

7. Acuity Adjustments

A. RATE DEVELOPMENT STANDARDS

This section provides information on any acuity adjustments in the MICH program.

B. APPROPRIATE DOCUMENTATION

Section I, item 2.B.iii.d describes acuity adjustments made to reflect expected differences in acuity within the rate cells between the base and rating periods.

Section II. Medicaid Managed care rates with long-term services and supports

1. Managed Long-Term Services and Supports

A. SECTION I APPLICABILITY

The capitation rates developed for the LTSS portion of the MICH program are consistent with all requirements outlined in Section I of this report.

B. RATE DEVELOPMENT STANDARDS

The MICH capitation rates were developed based on a non-blended approach considering the long-term care setting that the beneficiaries are using.

C. APPROPRIATE DOCUMENTATION

i. Capitation Rate Structure

(a) Capitation Rate Structure

The MICH rate structure for calendar year 2026 is generally consistent with the rate structure under the prior MI Health Link program. Rates continue to vary by region consistent with current geographic definitions. The rate cells continue to represent population groups split by place of setting and represent a non-blended rate cell structure.

Nursing Facility

The Nursing Facility category represents MICH eligible members (as outlined earlier) who meet nursing facility level of care and reside in a facility. Separate rates were established based on age (Over/Under 65) and the type of facility (Private or County-Owned). HIDE SNPs will receive the Nursing Facility rate for beneficiaries who enroll into MICH program from the nursing facility setting.

Transition Rules

Members who had met the criteria for inclusion in the Nursing Facility cell, but later do not, will be transitioned to the community or waiver category. The HIDE SNP will immediately receive the alternative category rate upon switching from the Nursing Facility.

NFLOC-Waiver

The waiver category represents eligible dual members who meet the NFLOC standard (including the transition rules), but do not reside in a facility. Members cannot be dually enrolled in MICH and the state's MI Choice program simultaneously.

Community

The community category represents eligible dual members who do not meet the NFLOC standard. Within the community well category, capitation rates vary by region and age (Over/Under 65).

ii. Data, Assumptions, and Methodology

The description of the rates and rate cells, data, assumptions, and methodology are detailed in Section I of the report.

iii. Non-benefit costs

The description of the non-benefit costs of the rates and rate cells, data, assumptions, and methodology are detailed in Section I of the report.

iv. Experience and Assumptions

Section I details the experience and assumptions employed for the LTSS and non-MLTSS services included in the MICH program.

Section III. New adult group capitation rates

Section III of the guidance is not applicable to the MICH program as these are not new adult groups.

Limitations

The information contained in this letter, including the appendices, has been prepared for the State of Michigan, Department of Health and Human Services and their consultants and advisors. It is our understanding that this letter may be utilized in a public document. To the extent that the information contained in this letter is provided to third parties, the letter should be distributed in its entirety. Any user of the data must possess a certain level of expertise in actuarial science and healthcare modeling so as not to misinterpret the data presented.

Milliman makes no representations or warranties regarding the contents of this report to third parties. Likewise, third parties are instructed that they are to place no reliance upon this report prepared for MDHHS by Milliman that would result in the creation of any duty or liability under any theory of law by Milliman or its employees to third parties. Other parties receiving this report must rely upon their own experts in drawing conclusions about the capitation rates, assumptions, and trends.

Milliman has developed certain models to estimate the values included in this report. We have reviewed the models, including their inputs, calculations, and outputs for consistency, reasonableness, and appropriateness to the intended purpose and in compliance with generally accepted actuarial practice and relevant actuarial standards of practice (ASOP). The models rely on data and information as input to the models. We have relied upon certain data and information provided by MDHHS and the participating HIDE SNPs for this purpose and accepted it without audit. To the extent that the data and information provided is not accurate, or is not complete, the values provided in this report may likewise be inaccurate or incomplete. The models, including all input, calculations, and output may not be appropriate for any other purpose.

It should be emphasized that capitation rates are a projection of future costs based on a set of assumptions. Differences between our projections and actual amounts depend on the extent to which future experience conforms to the assumptions made for this analysis. It is certain that actual experience will not conform exactly to the assumptions used in this analysis. Actual amounts will differ from projected amounts to the extent that actual experience deviates from expected experience.

Guidelines issued by the American Academy of Actuaries require actuaries to include their professional qualifications in all actuarial communications. The authors of this report are members of the American Academy of Actuaries and meet the qualification standards for performing the analyses contained herein.

Appendix 1: Actuarial Certification

State of Michigan, Department of Health and Human Services
MI Coordinated Health Program
Calendar Year 2026 Medicaid Component Capitation
Actuarial Certification

I, Christopher Pettit, am a Principal and Consulting Actuary with the firm of Milliman, Inc. I am a Member of the American Academy of Actuaries and a Fellow of the Society of Actuaries. I meet the qualification standards established by the American Academy of Actuaries and have followed the standards of practice established by the Actuarial Standards Board. I have been retained by the State of Michigan, Department of Health and Human Services, to perform an actuarial review and certification regarding the development of capitation rates for the MI Coordinated Health program effective January 1, 2026. I am generally familiar with the state-specific Medicaid program, eligibility rules, and benefit provisions.

The capitation rates provided with this certification are considered "actuarially sound" for purposes of 42 C.F.R. § 438.4(a), according to the following criteria:

- the capitation rates provide for all reasonable, appropriate, and attainable costs that are required under terms of the contract and for the operation of the MCO for the time period and population covered under the terms of the contract, and such capitation rates were developed in accordance with the requirements under 42 C.F.R. § 438.4(b).

For the purposes of this certification and consistent with the requirements under 42 C.F.R. § 438.4(a), "actuarial soundness" is defined as in ASOP 49:

"Medicaid capitation rates are "actuarially sound" if, for business for which the certification is being prepared and for the period covered by the certification, projected capitation rates and other revenue sources provide for all reasonable, appropriate, and attainable costs. For purposes of this definition, other revenue sources include, but are not limited to, expected reinsurance and governmental stop-loss cash flows, governmental risk-adjustment cash flows, and investment income. For purposes of this definition, costs include, but are not limited to, expected health benefits; health benefit settlement expenses; administrative expenses; the cost of capital, and government-mandated assessments, fees, and taxes."

The assumptions used in the development of the "actuarially sound" capitation rates have been documented in my correspondence with the State of Michigan. The "actuarially sound" capitation rates that are associated with this certification are effective for calendar year 2026.

The "actuarially sound" capitation rates are based on a projection of future events. Actual experience may be expected to vary from the experience assumed in the rates.

In developing the "actuarially sound" capitation rates, I have relied upon data and information provided by the State. I have relied upon the State for audit of the data. However, I did review the data for reasonableness and consistency.

The capitation rates developed may not be appropriate for any specific health plan. An individual health plan will need to review the rates in relation to the benefits that it will be obligated to provide. The health plan should evaluate the rates in the context of its own experience, expenses, capital and surplus, and profit requirements prior to agreeing to contract with the State of Michigan. The health plan may require rates above, equal to, or below the "actuarially sound" capitation rates that are associated with this certification.



Christopher T. Pettit, FSA
Member, American Academy of Actuaries

December 12, 2025
Date

Appendix 2: CY 2026 Cost Models (Provided Separately in Excel)

Appendix 3: CY 2026 Capitation Rate Development (Provided
Separately in Excel)

Appendix 4: Trend Analysis (Provided Separately in Excel)



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